Week #8: [Osteology & Joints of the Proximal Upper Extremity]

Osteology of the Proximal Upper Extremity

Joints of the Proximal Upper Extremity

Approach to Acromioclavicular & Glenohumeral Joint Problems, Shoulder Instability, Adhesive Capsulitis (Dr. Faber)

Osteology of the Proximal Upper Extremity



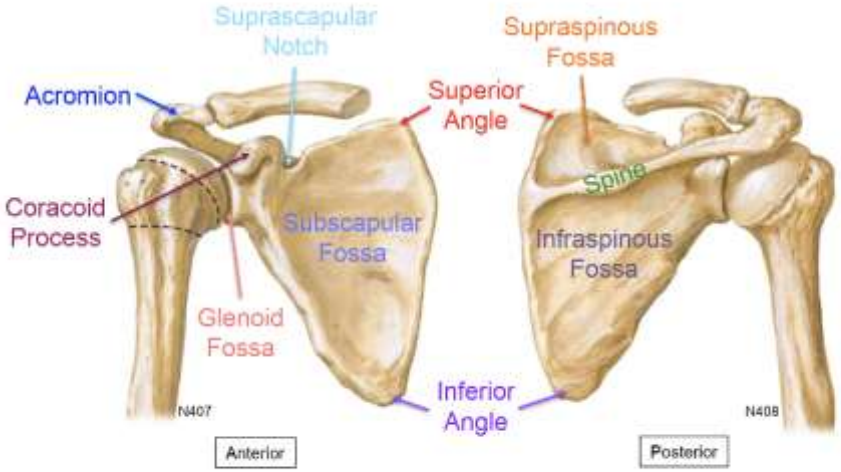
Osteology:

a. The pectoral girdle consists of the clavicle and the scapula. There are four joints that comprise the pectoral girdle
   i. **Sternoclavicular joint** – between the proximal clavicle and the manubrium of the sternum
   ii. **Acromioclavicular joint** - between the distal clavicle and the acromion process of the scapula
   iii. **Scapulothoracic joint** – between the deep aspect of the scapula and the thoracic wall
   iv. **Glenohumeral joint** – between the glenoid fossa of the scapula and the head of the humerus

Clavicle – Forms an S-shape with its medial half convex anteriorly and the lateral half concave anteriorly

a. The medial end that articulates with the manubrium is enlarged and triangular while the acromial end is more flat where it articulates with the acromion process

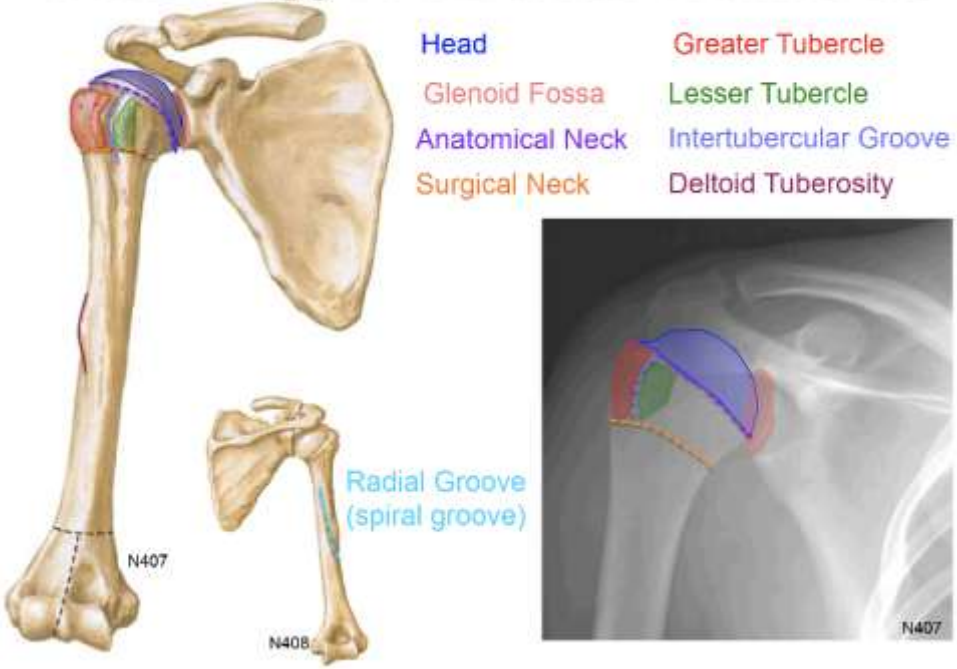
Osteology: Scapula



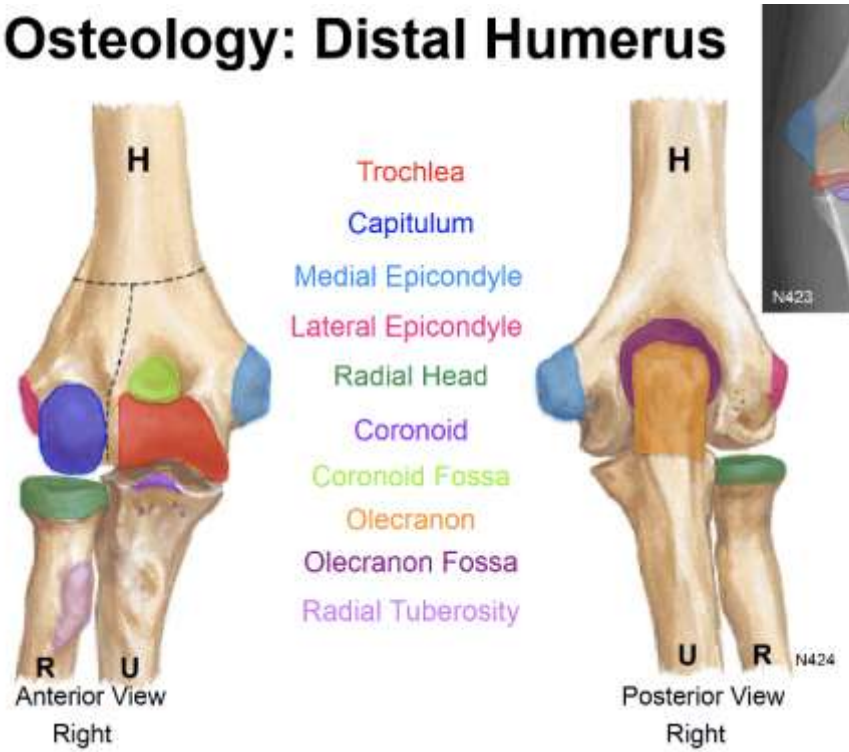
Scapula – triangular flat bone that is essential for overhead movements of the upper extremity

1. **Acromion Process** – articulates with the clavicle to form the acromioclavicular joint
2. **Coracoid Process** – large projection for muscle attachments
3. **Glenoid Fossa** – shallow depression that articulates with the head of the humerus to form the glenohumeral joint
    a. ***Supraglenoid Tubercle*** – projection at the superior aspect of the glenoid fossa for the origin of the long head of the biceps brachii
    b. ***Infraglenoid Tubercle*** – projection at the inferior aspect of the glenoid fossa for the origin of the long head of the triceps brachii
4. **Superior Angle and Inferior Angle** – sharp ridges for muscle attachments and landmarking
5. **Subscapular Fossa** – concavity in which the subscapularis muscle lies
6. **Spine of the scapular** – divides the posterior scapula into the supraspinous fossa and the infraspinous fossa
    a. The ***supraspinatus muscle*** lies within the supraspinous fossa
    b. The ***infraspinatus muscle*** lies within the infraspinous fossa
7. **Suprascapular Notch** – forms the suprascapular foramen when the transverse scapular ligament crosses it. Neurovascular structures use this notch to travel to and from the anterior aspect of the scapula

Osteology: Proximal Humerus



Osteology: Distal Humerus



Humerus – only bone of the arm

1. **Head** – articulates with the scapula at the glenohumeral joint
2. **Greater tubercle** – prominence on the superolateral humerus; insertion site
3. **Lesser tubercle** – prominence on the superoanterior humerus; insertion site
4. **Intertubercular (bicipital) groove** – between greater and lesser tubercles; tendon of ***long head of biceps brachii*** runs through groove
5. **Anatomical neck** – separates head from tubercles
6. **Surgical neck** – narrowing distal to head and tubercles; common fracture site
7. **Radial (spiral) groove** – located on posterior shaft; ***radial nerve and profunda brachii artery*** run along groove
8. **Medial epicondyle** – medial protuberance at the distal end of the humerus; origin of the ***common flexor tendon for the forearm flexors***; ulnar nerve courses posterior to it
9. **Lateral epicondyle** – lateral protuberance at the distal end of the humerus; origin of the ***common extensor tendon for the forearm extensors***
10. **Capitulum** – lateral condyle; articulates with the head of the radius
11. **Trochlea** – medial condyle; articulates with the trochlear notch of the ulna
12. **Coronoid fossa** – depression superior to the trochlea anteriorly; articulates with the coronoid process during full flexion
13. **Olecranon fossa** – depression superior to the trochlea posteriorly; articulates with the olecranon process during full extension

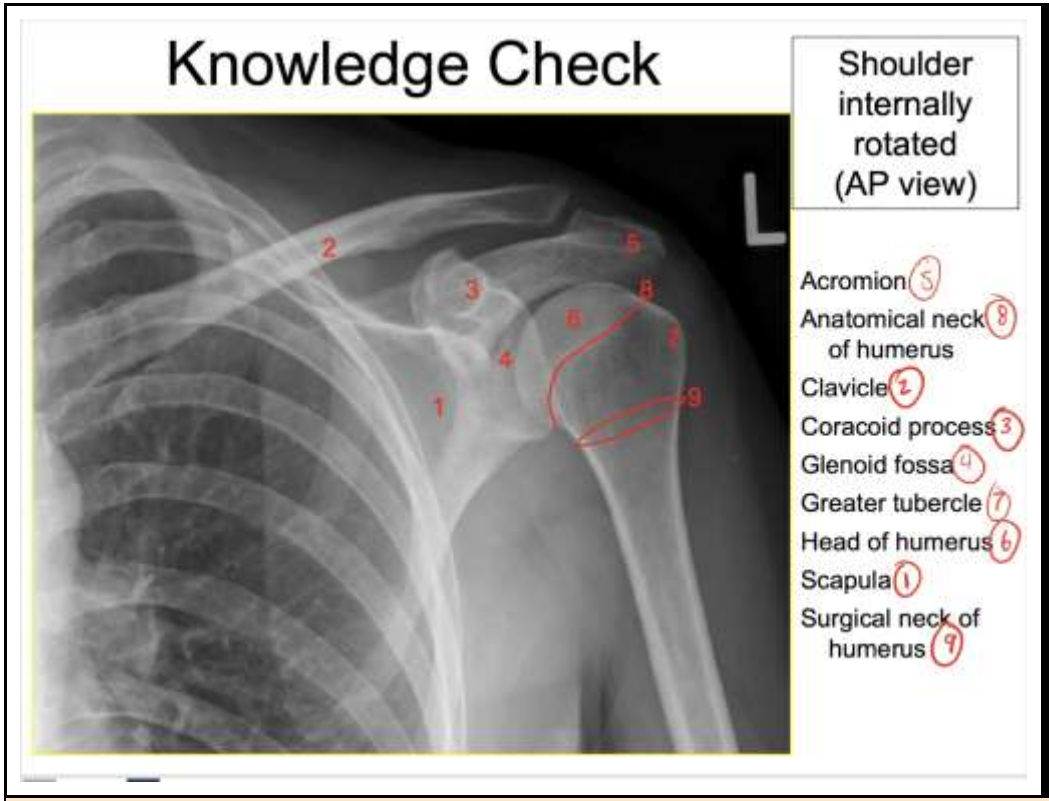
Clinical Note: Fractures of the **surgical neck** of the humerus, often caused by low energy falls to the hand are especially common in elderly individuals with osteoporosis. This type of fracture may result in damage to the ***axillary nerve.*** Transverse fractures of the shaft of the humerus frequently result from a direct blow to the arm. This may result in injury to the ***radial nerve*** and/or ***deep brachial artery***. Fractures of the distal humerus may damage the **median nerve.** Fractures of the ***medial epicondyle*** may result in damage to the ***ulnar nerve.***

Radius – lateral and shorter bone of the forearm

1. **Head** – proximal portion that articulates with the capitulum of the humerus and radial notch of the ulna; pivots within the ***annular ligament***
2. **Neck** – narrow portion between the head and the radial tuberosity
3. **Radial tuberosity** - medial projection; insertion site for ***biceps brachii***

Ulna – medial and longer stabilizing bone of the forearm.

1. **Olecranon process** – bony eminence at posterior aspect of proximal ulna; insertion site for ***triceps brachii***
2. **Coronoid process** – triangular protuberance at anterior aspect of proximal ulna; insertion site of ***brachialis***
3. **Trochlear notch** – large depression between olecranon and coronoid process; articulates with trochlea
4. **Radial notch** – concavity on lateral side of coronoid process that articulates with the head of the radius

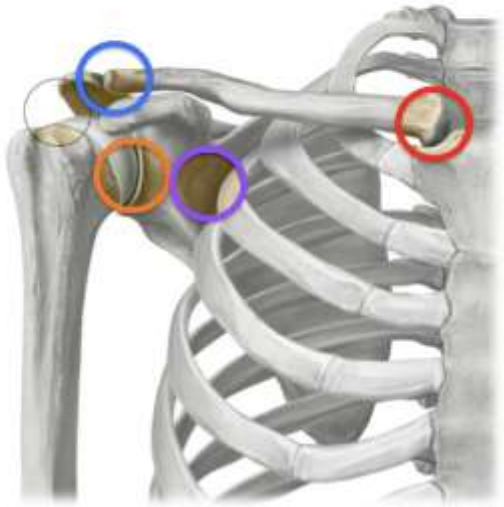


Joints of the Proximal Upper Extremity

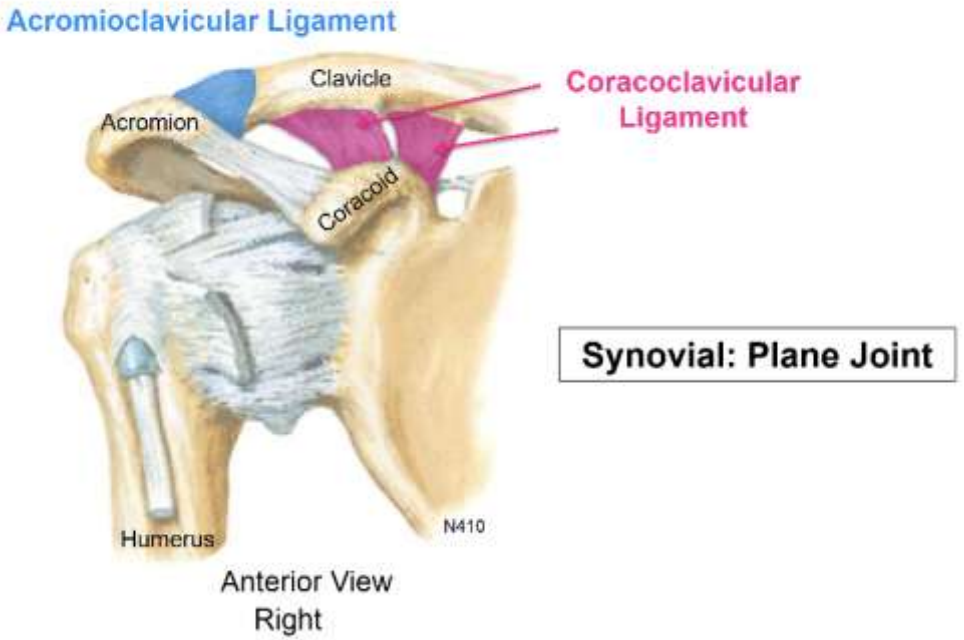
- **AC joint**
- **Glenohumeral (GH) Joint**

Pectoral Girdle Joints

- 4 Joints
  1. Sternoclavicular
  2. Acromioclavicular
  3. Glenohumeral
  4. Scapulothoracic



Acromioclavicular (AC) Joint



Joints of the Pectoral Girdle

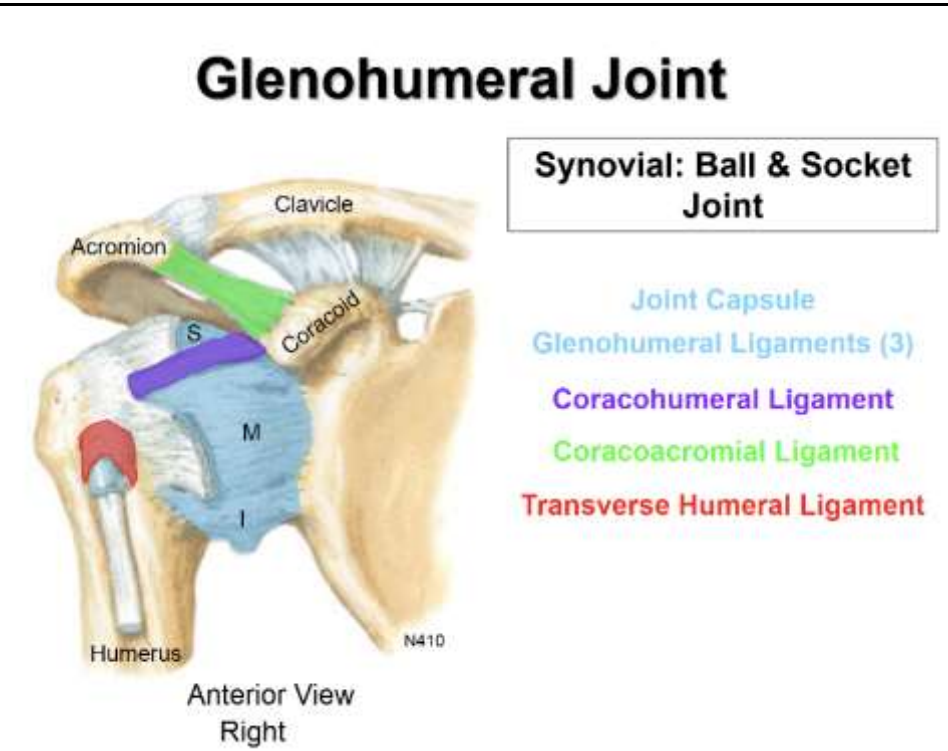
Scapulohumeral Rhythm: Full range of motion at the shoulder requires manipulation of the glenoid cavity position/angle; for muscles to function efficiently, their origins must be stable.

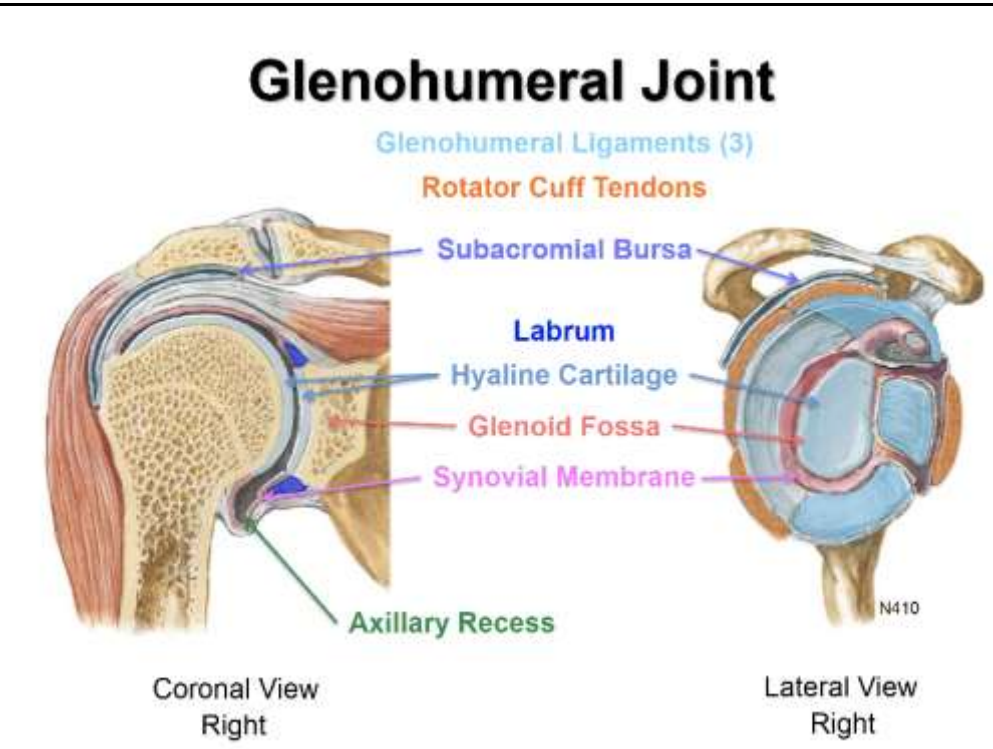
A. Sternoclavicular (SC) joint

- Only articulation conjoining the upper extremity to the axial skeleton.
- **Clinical Note:** Dislocations of the sternoclavicular joint are rare due to the strength of the surrounding ligaments. The clavicle is likely to fracture before the joint dislocates.

B. Acromioclavicular (AC) joint

1. ***Type:*** plane type of synovial joint; uniaxial
2. ***Articulation:*** acromial end of clavicle, acromion of scapula
   a. **Note:** joint space contains wedge-shaped articular disc
3. ***Ligaments:***
   a. Acromioclavicular ligament: strengthens joint superiorly
   b. Coracoclavicular ligament: pair of strong bands that anchor the clavicle to the coracoid process; confers great deal of stability/strength to the joint; enables the scapula and free limb to be passively suspended from the strut-like clavicle.
4. ***Movements:*** the acromion of the scapula rotates on the acromial end of the clavicle
5. **Clinical Note:** dislocation of the AC joint can result from direct or indirect force upon the joint. AC subluxations are usually associated with upward forces along the humerus and result in the rupture of the acromioclavicular ligament. AC dislocation is deemed more severe and is associated with blunt force trauma to the tip of the shoulder and results in the rupture of the strong, coracoclavicular ligament along with the acromioclavicular ligament; patient may present with clavicle displaced superiorly and acromion more prominent upon palpation.





C. Glenohumeral (shoulder) joint

1. ***Type:*** **ball and socket** type of synovial joint; multiaxial
2. ***Articulation:*** large **humeral head;** shallow **glenoid fossa of scapula**
    a. **Mobility** is enhanced by (i) *poor articular congruency*; (ii) *several bursae;* (iii) *high degree of articular cartilage;* and (iv) *lax joint capsule.*
    b. **Stability i**s enhanced by the ring-like, fibrocartilaginous glenoid labrum (increases fit, deepens socket), the **musculotendinous rotator cuff**, and **ligament support.**
    c. The upper limb **range-of-motion** of the shoulder involves not only the **glenohumeral joint,** but also involves movement of the **SC and AC joints** and movement of the scapula on the thorax (conceptual '**scapulothoracic joint**').
3. ***Ligaments:***
    a. **Glenohumeral ligaments** (superior, middle, inferior): thickenings of the joint capsule that strengthen the anterior aspect of capsule
    b. **Coracohumeral ligaments:** a strong band that passes from the base of the coracoid process to the anterior aspect of the greater tubercle; strengthens the capsule superiorly.
    c. **Coracoacromial ligament:** protective structure spanning from acromion to coracoid process of scapula (inferior aspect of these bony structures and ligament termed coracoacromial arch); prevents superior displacement.
    d. **Transverse humeral ligament:** runs from greater tubercle to lesser tubercle over intertubercular sulcus/groove that provides passage for long head of biceps brachii tendon

Clinical Note: The **coracoacromial arch** is so strong, extremely forceful superior thrust would sooner fracture the humerus or clavicle before fracturing the arch.

- ***Movements:*** most mobility of all joints in the body; review muscles from shoulder lecture.
- ***Blood supply:*** **anterior** and **posterior circumflex humeral a., suprascapular a.**
- ***Innervation:*** **Hilton's Law**: suprascapular n., axillary n., lateral/medial pectoral n., musculocutaneous n., upper/lower subscapular n., thoracodorsal n., radial n.

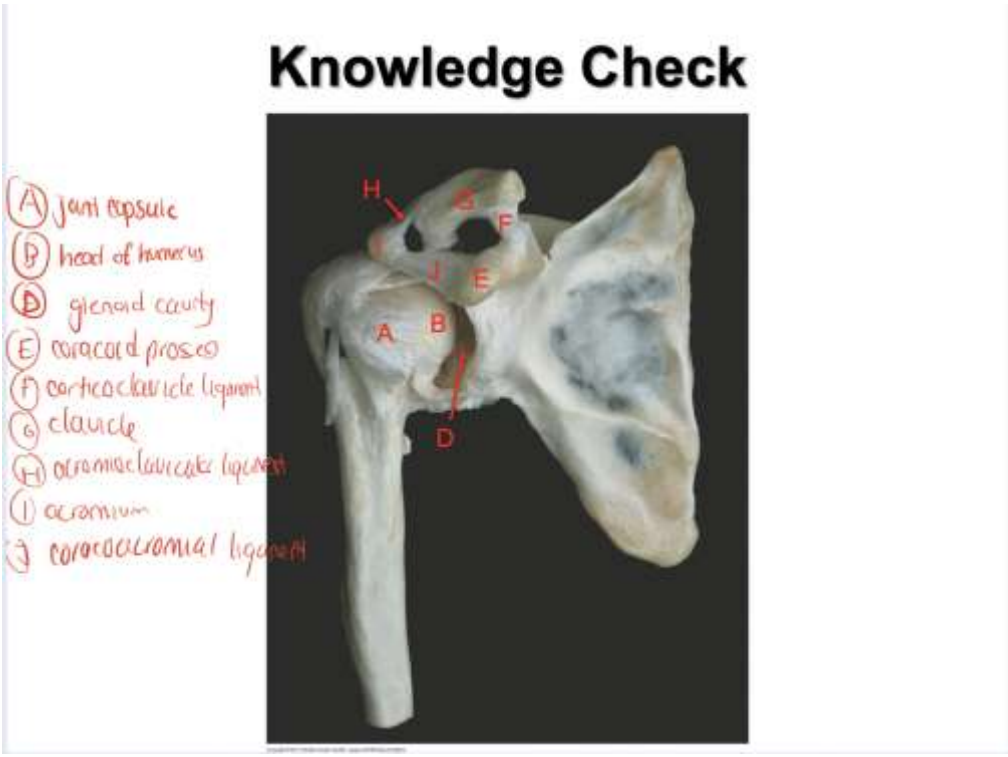
Clinical Note: dislocation of the glenohumeral joint typically happens **inferiorly** and **anteriorly (**due to the coraco-acromial arch superiorly and the **weakness of the joint capsule** antero inferiorly); the **axillary nerve** is at risk during typical dislocation.

- Treatment regimen usually includes immobilization and therapy to strengthen the rotator cuff muscles.

Clinical Note: **Shoulder Impingement/Instability**. **Shoulder Impingement Syndrome** is caused when the tendon of the **supraspinatus** becomes impinged in the subacromial space leading to tendonitis of the tendon.

- Chronic overuse of the glenohumeral joint may stretch the static stabilizers of the shoulder causing mild shoulder instability.
- Instability puts increased stress on the rotator cuff to contain the humerus in the center of the glenoid fossa. Rotator cuff muscles become weakened leading to a subluxation of the glenohumeral joint resulting in a secondary **impingement** of supraspinatus in the subacromial space.





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